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A FIELD GUIDE FROM PERSEVERANT DIGITAL · FOR ELECTIVE HEALTHCARE

The Cost-per-Appointment Blueprint for Meta Ads

Work out what you can afford to pay for a booked appointment, choose an offer worth raising a hand for, and set up Meta lead campaigns judged on appointments, not cheap leads.

THE SHORT ANSWER · 65 WORDS

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Facebook and Instagram ads can produce real appointments for a high-value service business when three things line up. You know the most you can pay per booked appointment, you lead with one offer worth raising a hand for, and someone calls every lead within minutes. Judge the campaign on booked and held appointments, because the cheapest leads are often the least likely to show up.

Many owners who say Facebook ads “don’t work” are judging them on the wrong number. A cheap lead goes to voicemail, so the channel looks like junk. An expensive lead gets the campaign switched off before it books anyone.

This guide gives you a better scoreboard: the most you can pay for a booked appointment. Everything else is built to hit it.

Step 1: What can you afford to pay for a booked appointment?

Your ceiling comes from your own numbers, not an industry average. Use the last 12 months. Where you don’t track a number, use an honest guess and mark it.

Line	Your number
Average first-year value of a new client	$
× Gross margin (%) = value of a new client	$
× Close rate from a held appointment (%) = value of a held appointment	$
× Show rate (%) = value of a booked appointment	$
× Share you'll spend on acquisition (%)	
= Maximum cost per booked appointment	**$**

Use first-year value, not lifetime value. It keeps you safe while you learn what Meta leads actually do.

A worked example. Example numbers, not benchmarks.

- $8,000 first-year value × 50% margin = $4,000 per new client
- $4,000 × 25% close rate = $1,000 per held appointment
- $1,000 × 70% show rate = $700 per booked appointment
- $700 × 30% spend share = **$210 maximum cost per booked appointment**

Now work backward. If 40% of leads book, this business can pay up to $84 per lead. If only 10% book, it needs leads under $21. A $30 lead that books half the time is cheaper than a $12 lead that almost never does.

- [ ] Fill in the worksheet and circle every guess
- [ ] Post your maximum cost per booked appointment where your team can see it

What this looks like for elective practices

Elective patients are interested but rarely in a hurry. Someone weighing implants, a facelift or LASIK may watch your videos for months. A campaign works when it offers a low-pressure next step and someone answers quickly when they take it.

Demand is there. The American Society of Plastic Surgeons reported cosmetic surgical procedure volume rose 7% in 2025, with facelifts up 13% (ASPS, September 1, 2026).

Meta makes it harder than it used to be. Its 2025 restrictions on health and wellness advertisers can block optimization toward lower-funnel events such as scheduling, while in-app lead forms were affected least (Search Engine Land, March 11, 2025). That makes Instant Forms a practical start.

Privacy is the line you don't cross. Ask about timing and interest, never conditions or history. Keep anything that could be health information out of Meta, analytics and texts.

Step 2: Which offer is worth raising a hand for?

Pick one offer per campaign. It should match how ready the prospect is and never promise a result.

Offer	Fits	Keep it clean
Consultation with the provider	Implants, plastic surgery, LASIK	No promised results
Open house or seminar	Plastic surgery, med spa	Describe the evening, not the outcome
"What recovery really looks like" guide	Plastic surgery, implants	General information only
Cost and financing explainer	Ortho, implants	Clear ranges, no "starting at" bait

- [ ] Would a serious prospect find this useful even if they never hire us?
- [ ] Can we deliver it well at the volume a campaign could send?
- [ ] Is the wording free of promised outcomes or savings figures?

Step 3: Lead form or landing page?

Start with Meta's Instant Form set to Higher intent, add one or two multiple-choice qualifying questions, and move to a landing page only if lead quality stays low. Higher intent adds a review screen before submitting. It costs some volume and screens out some accidental taps.

Option	Use it when	Watch out for
Instant Form, More volume	Your team can call everyone fast	Accidental submissions, wrong numbers
Instant Form, Higher intent	You want fewer, more deliberate leads	Slightly higher cost per lead
Either form plus qualifying questions	You need to route by location, timeline or service	More than two questions hurts completion
Landing page	The offer needs explaining, or you want booking on the spot	Needs fast load, clean tracking, short form

Ask only what the first call needs.

Never ask about a diagnosis, condition or treatment history in an ad platform's form. Ask which service interests them and when to talk.

Step 4: What are the rules for your category?

Check your category before you build. This is a factual summary, not legal advice.

- **Health and wellness.** Meta's 2025 restrictions can block optimization toward scheduling; lead forms were affected least (Search Engine Land, March 11, 2025).
- **HIPAA tracking.** Tracking on scheduling pages and other pages that transmit protected health information is still covered, and vendors handling it need a business associate agreement (HHS OCR guidance).
- **Texts and calls (everyone).** Prior express written consent is still required for autodialed or prerecorded marketing texts and calls, and since April 11, 2025 opt-outs by any reasonable means must be honored within 10 business days (Kelley Drye; Burr & Forman).
- **Testimonials (everyone).** The FTC's August 2024 rule bans fake, bought and undisclosed insider reviews (Morgan Lewis, August 26, 2024).

Step 5: How do you test creative in 14 days?

Run three hooks across two formats, keep the offer, audience and form fixed, and judge on booked appointments. The hook, the first line or first two seconds, is usually your biggest lever.

	Short video, talking to camera	Single image with text
Hook 1: the problem in the prospect's words		
Hook 2: what happens at the appointment		
Hook 3: a common mistake or myth		

- [ ] Days 1–7: Launch all six and leave them alone. Pause early only if an ad spends well past your target with zero leads.
- [ ] Days 8–14: Compare booked appointments by ad. Pause clear losers, keep the top one or two.
- [ ] Day 15: Write down what you learned and change one thing for the next round.

Cost per booked appointment decides. Cost per lead and click-through rate only help you diagnose.

Step 6: What happens after someone fills out the form?

Call every lead within minutes and track each one to a result. This decides more than any ad does. In a Harvard Business Review data set of 1.25 million leads, firms that made contact within an hour were nearly seven times as likely to qualify the lead as firms that waited longer (HBR, March 2011).

Let automation send the confirmation text. A person should make the call.

Date	Name	Ad or hook	Minutes to call	Reached?	Booked?	Held?	Won?	Value

Each week, divide spend by booked appointments and compare it to your Step 1 ceiling.

If you later send outcomes back to Meta, a "booked" event must say nothing about why the person booked.

- [ ] Name who owns new leads at every hour you advertise
- [ ] Add an "ad or hook" field to every lead record

Where do most owners get stuck?

> **Sending CRM outcomes back to Meta correctly.** Meta's Conversions API can learn from "booked" and "held" events, but only if lead IDs match, timing is right and nothing sensitive goes along. Get it wrong and Meta learns the wrong lesson.

> **Making enough creative to beat fatigue.** Winning ads wear out. Most small teams run out of time for new hooks before the campaign runs out of steam.

> **Measuring without exposing patient data.** Linking a lead to a held consultation without leaking health information takes careful setup and a recheck after every website change.

If you've filled in the worksheet and want a second set of eyes on the numbers, book a 25-minute exploration call with the short form at the end of this guide.

Sources

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- U.S. Department of Health and Human Services, Office for Civil Rights, "Use of Online Tracking Technologies by HIPAA Covered Entities and Business Associates," accessed October 15, 2026. link
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- Burr & Forman, "The FCC Delays Effective Date of TCPA Revoke-All Rule Until January 31, 2027," accessed October 15, 2026. link
- Morgan Lewis, "FTC Issues Final Rule on Consumer Reviews and Testimonials," August 26, 2024. link
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- American Society of Plastic Surgeons, "Lift Procedures Led Growth as Demand for Restoration Drove Plastic Surgery in 2025," September 1, 2026. link

Want a second set of eyes on what you found?

Book a free 25-minute exploration call. We will look at your answers with you, name the first fixes in order, and tell you honestly if we are not the right fit.

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